



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 26, 2025
FSC: HMO/PPG
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 39
Cxl/Ns: 8

Daily Note

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
11/14: Patient reports that he is sore and the biceps are hurting still.
11/18: Patient reports that his shoulder feels better and the biceps is not hurting today.
11/20: The patient reports his shoulder is feeling pretty good and he was not too sore after the last session.
11/24: Pt reports feeling weird since the pop last session but feels like he gained more mobility with it
11/25: Patient reports that he is feeling better after last session.
11/28: Patient reports that his shoulder feels okay.
12/2: The patient reports his shoulder is feeling less sore and the motion continues to improve.
12/4: The patient reports his shoulder felt really good after the last session and he was not too sore. It is continuing to feel stronger.
12/5: Pt reports a good soreness in his L shoulder today after his HEP and last session.
12/11: The patient reports his shoulder feels a little more stiff in the past day or so.
12/17: Pt reports no pain in his L shoulder today. Pt states his goal is to improve in the next 2 weeks.
12/18: The patient reports his shoulder continues to do well, however feels stiff from doing more with exercises. It can still feel weak when he is doing prolonged overhead activities.
12/22: Pt reports shoulder still feeling stiff, sees Dr Grey tomorrow maybe for his last visit
12/26: Pt reports his shoulder is gaining strength and he is working on his HEP independently.

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

- Inferior Humeral Glide
- Posterior Humeral Glide

Left

Hypomobile/Painful
Hypomobile/Painful

Right

Normal
Normal

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left

-4/5
4/5
4/5
4/5

Right

5/5
5/5
5/5
5/5

Range of Motion: Shoulder: Pre-Treatment:

- Flexion
- Abduction
- External Rotation - 0 deg
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM	Right AROM	Right PROM
125	175	135	150
103	165	135	155
65			
75	95	35	65
30	60	45	45

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 4 Minutes, Description: PROM all ways, Charge As: Therapeutic Exercise, Performed by: Jessica j Uptmore, PTA, 4 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Additional Detail: and scar tissue passage, Time Elapsed: 5 Minutes, Tx Depth: Moderate, Technique: Rolling, Description: UT, LS, pec, delt, shoulder girdle/post cuff, Charge As: Manual Therapy Techniques, Performed by: Jessica j Uptmore, PTA, 5 Minutes.

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)
- Wall Walk/Finger Ladder(with leading in for over pressure stretch)

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Did Not Perform: with leading in for over pressure stretch

• Rope & Pulley Flexion

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

• T-Bar ER 45deg

Did Not Perform:

• ROM Activity 2

Did Not Perform:

Exercise Activities: Isotonics:

• Isotonic Activity 2

Did Not Perform:

Pt./Family Education:

• Patient Education 1

Time Elapsed: 2 Minutes, Description: ROM, joint versus mm restriction, goals, POC, Charge As: Therapeutic Activities, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Did Not Perform:

• Written Home Exercise Program

Manual Interventions: Joint Segments: Shoulder:

• Glenohumeral Joint

Time Elapsed: 5 Minutes, Technique 2: flex mobs towards end range, Technique 3: regular inferior glides passive, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Jessica j Uptmore, PTA, 5 Minutes.

Exercise Activities: Dynamic Training: C-K-C Activity:

• Wall Ball

Time Elapsed: 2 Minutes, Performed: GMB, Additional Detail: at shoulder height in abduction position and OH positions, Repetitions: 30 seconds each way, Direction: up/down, side/side, circles CW/CCW, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Did Not Perform:

• Closed Kinetic Chain 1

Did Not Perform:

• Closed Kinetic Chain 3

Exercise Activities: Aerobic Conditioning:

• Upper Body Ergometer

Direction: forward & backward, Speed (rpm): self selected, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 4 Minutes.

Exercise Activities: Stabilization Training: General:

• Stabilization Training 2:(Pt active assisted help with Y's during concentric phase)

Did Not Perform: Pt active assisted help with Y's during concentric phase

• Stabilization Training 3:

Did Not Perform:

• Stabilization Training 4:

Did Not Perform:

Exercise Activities: Tubing/Bands:

Exercise Activities: Machines/Weights:

• Low Rows

Additional Detail: single and double, Time Elapsed: 2 Minutes, Repetitions: 15, Sets: 2, Plates: 4/3, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

• Pull Downs: Sym. Bilateral

Time Elapsed: 2 Minutes, Repetitions: 10, Sets: 2, Plates: 5, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

• Machines/Free Weights 1

Time Elapsed: 2 Minutes, Repetitions: 10, Sets: 2, Description: standing punches with serratus press on cable machine, Plates: 3, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

• Push Downs: Unilateral

Did Not Perform:

• Machines/Free Weights 2

Did Not Perform:

• Machines/Free Weights 3

Weight - Pounds: #5 Pounds, Sets: 2 laps, Description: 90 flex carries with 90/90 movement walking, Time Elapsed: 1 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 1 Minutes.

Exercise Activities: Dynamic Training: TRX:

• TRX Customized Exercise 1

Repetitions: 15, Additional Detail: TRX Row, T, Y, Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Therapy Session Time

• Therapy Session Start Time	12/26/2025 10:30 AM
• Therapy Session Stop Time	12/26/2025 11:05 AM
• Total Therapy Session Time	35 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	10
• Neuromuscular Reeducation	97112	1.00	15
• Therapeutic Activities	97530	1.00	6
• Therapeutic Procedure	97110	1.00	4

Assessment

Pt warmed up on UBE and pulleys to perform 90/90 flexion hallway carries and low rows with minimal fatigue. Pt continues to benefit from skilled PT to progress strength with functional shoulder mobility to return to PLOF.

Plan

Left Shoulder

Daily Plan: Continue w/ Current Rehabilitation Program.



Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
Signed on Dec 28, 2025 08:23:15



Jessica j. Uptmore, PTA, PTA(CO Lic: PTA.0015772)
Signed on Dec 27, 2025 16:08:49



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Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Sep 02, 2025
FSC: HMO/PPO
Payor: AETNA

Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 1
Cxl/Ns: 0

Initial Evaluation

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill Sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

History: Medical: Past/Present: COVID-19.

The medical history questionnaire including fall risk screening has been completed and signed by the patient, reviewed by the evaluating therapist, and is on file.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

History: Medical: Functional Comorbidity Index: Upper Gastrointestinal Disease.

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Premorbid Status: ADL/ IADL: Independent Without Difficulty.

History: Medical: Functional Comorbidity Index: Body Measurements:

- Height (in) 72 Inches
- Weight (lbs) 140 Pounds

• Body Mass Index - Calculated 18.99

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying unable
- Dressing unable to don/doff shirts independently
- Housework- Light unable
- Lifting unable
- Personal Hygiene Requirements unable to clean axillary area
- Reaching unable
- Sleeping spasms preventing him from falling asleep

Objective Examination

Left Shoulder

Observations: Cardiovascular System: Pre-Treatment:

- Blood Pressure: Systolic 112 mm/Hg
- Blood Pressure: Diastolic 75 mm/Hg

Joint Integrity/Mobility: Shoulder:

- | | | |
|---------------------------|--------------------|--------------|
| | Left | Right |
| • Inferior Humeral Glide | Hypomobile/Painful | Normal |
| • Posterior Humeral Glide | Hypomobile/Painful | Normal |

Muscle Testing: Upper Extremity MMT:

- | | | |
|-------------------------------|-------------|--------------|
| | Left | Right |
| • Shoulder Abduction:(0= NT) | 0/5 | 5/5 |
| • Shoulder Flexion: | 0/5 | 5/5 |
| • Shoulder External Rotation: | 0/5 | 5/5 |
| • Shoulder Internal Rotation: | 0/5 | 5/5 |

Range of Motion: Shoulder: Pre-Treatment:

- | | | | | |
|------------------------------|------------------|------------------|-------------------|-------------------|
| | Left AROM | Left PROM | Right AROM | Right PROM |
| • Flexion(0=NT) | 0 | 40 | 135 | 150 |
| • Abduction | 0 | 35 | 135 | 155 |
| • External Rotation - 90 deg | 0 | 15 | 35 | 65 |
| • Internal Rotation - 90 deg | 0 | 5 | 45 | 45 |

Treatments

Left Shoulder

Pt./Family Education:

- Pathology/Involved Anatomy

Time Elapsed: 8 Minutes, Additional Detail: Discussed precautions, sling wear, hygiene, Charge As: Self Care / Home Mgt Training, Performed by: Layne Martin, PT, 8 Minutes.
- Written Home Exercise Program

Activity: elbow bends, pendulums, Description: Diagnosis Specific, Time Elapsed: 8 Minutes, Charge As: Therapeutic Activities, Performed by: Layne Martin, PT, 8 Minutes.

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1(to tolerance)

Time Elapsed: 8 Minutes, Description: PROM in all directions, Charge As: Therapeutic Exercise, Performed by: Layne Martin, PT, 8 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Tx Depth: Moderate, Technique: Rolling, Assisting Technique: and grade 1-2 jt glides for pain relief, Description: UT, LS, pec, delt, shoulder girdle, Time Elapsed: 8 Minutes, Charge As: Manual Therapy Techniques, Performed by: Layne Martin, PT, 8 Minutes.

Exercise Activities: Range of Motion:

- Pendulum

Time Elapsed: 4 Minutes, Repetitions: 20xs ea, Additional Detail: side/side, front/back, circles CW, CCW, Charge As: Therapeutic Exercise, Performed by: Layne Martin, PT, 4 Minutes.

- ROM Activity 1

Technique: Passive, Description: elbow bends, Time Elapsed: 4 Minutes, Charge As: Therapeutic Exercise, Performed by: Layne Martin, PT, 4 Minutes.

Exercise Activities: Isotonics:

Therapy Session Time

- Therapy Session Start Time 2025/09/02 21:04:00
- Therapy Session Stop Time 09/02/2025 09:59 PM
- Total Therapy Session Time 55 Minutes

Modalities:

- Cryotherapy Location: Anterior/Posterior, Clinical Use: Post Activity, Time Elapsed: 10 Minutes, Charge As: Hot or Cold Packs, Performed by: Layne Martin, PT, 10 Minutes.

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Evaluation, Moderate Complexity	97162	1.00	n/a
• Hot or Cold Packs	97010	1.00	n/a
• Manual Therapy Techniques	97140	1.00	8
• Self Care/Home Management Training	97535	1.00	8
• Therapeutic Activities	97530	1.00	8
• Therapeutic Procedure	97110	2.00	16

Assessment

Pt. presents to PT with limited ROM, strength and inability to use their left arm in any capacity due to recent surgery and current restriction from immobilization. Pt. does fair with PROM in clinic today but is limited with pain and guarding due to recent post operative status and extended period of per operative immobilization. They have fair form exercises. Pt. also has been completing sling use as prescribed.

The patient requires skilled physical therapy to address the problems identified, and to achieve the individualized patient goals as outlined in the problems and goals section of this evaluation. Overall rehabilitation potential is excellent.

The patient was educated regarding their diagnosis, prognosis, related pathology & plan of care. The patient demonstrates an excellent understanding of the risks, benefits, precautions/contraindications, & prognosis of their skilled rehab program.

Recommendations: Skilled Intervention:

- **Required To:** Decrease Pain. Improve Function. Improve Motor Control. Increase Range of Motion. Increase Strength.

Problems & Goals

Left Shoulder

Problem #1 Chief Complaint: Pain: Severity:

LTG Achieve by Sep 30, 2025. worst

Symptomatic Improvements: Decreasing Pain: to 2/10.

Problem #2 Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

LTG Achieve by Dec 31, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 35.00

STG Achieve by Nov 01, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 50.00

Problem #3 Muscle Testing: Upper Extremity MMT:

LTG Achieve by Dec 31, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

- Gross Assessment **Left** 5/5

STG Achieve by Nov 01, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

+3/5

Problem #4 Range of Motion: Shoulder: Pre-Treatment:

LTG Achieve by Dec 31, 2025.

Range of Motion Improvements to: Shoulder:

- Flexion
- Abduction
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM
145	160
145	160
85	90
45	50

STG Achieve by Nov 01, 2025.

Range of Motion Improvements to: Shoulder:

- Flexion
- Abduction
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM
120	145
125	145
70	90
25	35

Problem #5 ADL / Functional Status: Current Status: ADL/ IADL: Activities:

LTG Achieve by Dec 31, 2025.

ADL Improvements In: Activities:

- Carrying 7 - Complete independence
- Dressing 7 - Complete independence
- Housework- Light 7 - Complete independence
- Lifting 7 - Complete independence
- Reaching 7 - Complete independence
- Sleeping 7 - Complete independence

Plan

Left Shoulder

Amount, Frequency and Duration: **Frequency and Duration:** It is recommended that the patient attend rehabilitative therapy for 3 visits a week with an expected duration of 4 months. The outlined therapeutic procedures and services in the plan of care will address the problems and goals identified.

Therapeutic Contents: to include: Therex (97110), Neuroreed (97112), Manual(97140), Therapeutic Act (97530), Self care/Home (97535), Gait (97116), Group (97150), Heat/ice (97010), Estim (97014), US (97035), Dry needling (20560/20561), Traction (97012), Vaso (97016)



Layne Martin, PT, PT(CO Lic: PTL.0013452)
 Signed on Sep 02, 2025 16:29:32



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Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Sep 29, 2025
FSC: HMO/PPO
Payor: AETNA

Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 7
Cxl/Ns: 3

Re-Evaluation

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

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The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

	Initial Sep 02, 2025	Current Sep 29, 2025
• Current	3/10	^a 3/10
• At Worst	4/10	^a 4/10

^a Measured On Sep 02, 2025

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

	Initial Sep 02, 2025	Current Sep 29, 2025
• Date:	09/02/2025	^a 09/02/2025
• Assessment	Initial	^a Initial
• Disability/Symptom Score	77.00	^a 77.00

^a Measured On Sep 02, 2025

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

	Initial Sep 02, 2025	Current Sep 29, 2025
• Carrying	unable	^a unable

• Dressing	unable to don/doff shirts independently	^a unable to don/doff shirts independently
• Housework- Light	unable	^a unable
• Lifting	unable	^a unable
• Personal Hygiene Requirements	unable to clean axillary area	^a unable to clean axillary area
• Reaching	unable	^a unable
• Sleeping	spasms preventing him from falling asleep	^a spasms preventing him from falling asleep

^a Measured On Sep 02, 2025

Daily Comments: 9/4: was very sore after IE, it's a good sore/ache not painful/sharp in a bad way. Has been compliant with HEP and sling use. Has been trying to touch his stomach which is improving.

9/5: Pt reports the shoulder is still sore but the ice helps

9/8: Pt feeling really stiff today; elbow bends feel really good

9/11: Patient reports that his shoulder is tight.

9/29: Pt reports the shoulder is doing better but the bicep is still the most painful

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

Initial Sep 02, 2025		Current Sep 29, 2025	
Left	Right	Left	Right
• Inferior Humeral Glide	Hypomobile/Painful	^a Hypomobile/Painful	^a Normal
• Posterior Humeral Glide	Hypomobile/Painful	^a Hypomobile/Painful	^a Normal

^a Measured On Sep 02, 2025

Muscle Testing: Upper Extremity MMT:

Initial Sep 02, 2025		Current Sep 29, 2025	
Left	Right	Left	Right
• Shoulder Abduction:(0= NT)	0/5	^a 0/5	^a 5/5
• Shoulder Flexion:	0/5	^a 0/5	^a 5/5
• Shoulder External Rotation:	0/5	^a 0/5	^a 5/5
• Shoulder Internal Rotation:	0/5	^a 0/5	^a 5/5

^a Measured On Sep 02, 2025

Range of Motion: Shoulder: Pre-Treatment:

	Initial Sep 02, 2025				Current Sep 29, 2025				Change			
	L. Act.	L. Pas.	R. Act.	R. Pas.	L. Act.	L. Pas.	R. Act.	R. Pas.	L. Act.	Pas.	R. Act.	Pas.
• Flexion(0=NT)	0	40	135	150	^a 0	105	^a 135	^a 150		65		
• Abduction	0	35	135	155	^a 0	110	^a 135	^a 155		75		
• External Rotation - 90 deg	0	15	35	65	^a 0	32	^a 35	^a 65		17		
• Internal Rotation - 90 deg(not measured due to post-op restriction)	0	5	45	45	^a 0	0	^a 45	^a 45		-5		

^a Measured On Sep 02, 2025

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1(to tolerance)

Time Elapsed: 10 Minutes, Description: PROM in ER, flex, abd, Charge As: Therapeutic Exercise, Performed by: Kendell Baylor, PT, 10 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Additional Detail: and scar tissue passage, Time Elapsed: 8 Minutes, Tx Depth: Moderate, Technique: Rolling, Description: UT, LS, pec, delt, shoulder girdle/post cuff, Charge As: Manual Therapy Techniques, Performed by: Kendell Baylor, PT, 8 Minutes.

Exercise Activities: Range of Motion:

- ROM Activity 2 (passive flex and abd body moving on stationary arm)
 - Pendulum
 - ROM Activity 1
- Side: Bilateral, Technique: Active, Hold.: 10 seconds Seconds, Description: table slides, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 3 Minutes.
- Time Elapsed: 2 Minutes, Repetitions: 20xs ea, Additional Detail: side/side, front/back, circles CW, CCW, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 2 Minutes.
- Time Elapsed: 4 Minutes, Technique: Active, Description: elbow bends, Charge As: Therapeutic Activities, Performed by: Kendell Baylor, PT, 4 Minutes.

Exercise Activities: Isotonics:

Modalities:

- Cryotherapy

Did Not Perform:

Pt./Family Education:

- Patient Education 1

Additional Detail: Education on anatomy, tissue healing and the difference between PROM and AAROM, Time Elapsed: 4 Minutes, Description: Education on post-op restrictions and re-assessment, Charge As: Therapeutic Activities, Performed by: Kendell Baylor, PT, 4 Minutes.

Therapy Session Time

- Therapy Session Start Time
- Therapy Session Stop Time
- Total Therapy Session Time

09/29/2025 04:00 PM
 09/29/2025 04:36 PM
 36 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	8
• Neuromuscular Reeducation	97112	1.00	5
• Reevaluation	97164	1.00	n/a
• Therapeutic Activities	97530	1.00	8
• Therapeutic Procedure	97110	1.00	10

Assessment

Began with education on post-op precautions and pausing on pullys until cleared for AAROM by Dr Gray. Educated on avoiding IR and begin light scapular retraction in sling. Educated on STM to the bicep and scar. Incision healing well with no s/sx of infection. Patient has made very good progress with improved PROM and reduced pain rating scale, however still very limited due to post op restrictions, pain, and tissue restriction. Patient still requires skilled PT to address impairments, normalize deficits, and return to PLOF.

focused the remainder of session on PROM and scar tissue massage. Pt tolerated session well and is able to preform PROM with table slides as the body is moving away from the arm is a very passive flexion and abd.

Recommendations: Skilled Intervention: Required To: Decrease Pain.

The patient was educated regarding their diagnosis, prognosis, related pathology & plan of care. The patient demonstrates a good understanding of the risks, benefits, precautions/contraindications, & prognosis of their skilled rehabilitation program.

Recommendations: Skilled Intervention:

- **Required To:** Increase Range of Motion. Increase Strength.

Recommendations: Skilled Intervention: Required To: Increase Work Tolerance: Medium (up to 50 lbs Occ).

Recommendations: Skilled Intervention:

- **Improve Function.**
- **Improve Motor Control.**
- **Return to Premorbid State.**

- **Return To Work.**

Problems & Goals

Left Shoulder

Problem #1 Chief Complaint: Pain: Severity:

LTG Achieve by Sep 30, 2025. worst

Symptomatic Improvements: Decreasing Pain: to 2/10.

Problem #2 Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

LTG Achieve by Dec 31, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 35.00

STG Achieve by Nov 01, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 50.00

Problem #3 Muscle Testing: Upper Extremity MMT:

LTG Achieve by Dec 31, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

5/5

STG Achieve by Nov 01, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

+3/5

Problem #4 Range of Motion: Shoulder: Pre-Treatment:

LTG Achieve by Dec 31, 2025.

Range of Motion Improvements to: Shoulder:

- Flexion
- Abduction
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM
145	160
145	160
85	90
45	50

STG Achieve by Oct 27, 2025. Progress: Some progress.

Range of Motion Improvements to: Shoulder:

- Flexion
- Abduction
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM
120	145
125	145
70	90
25	35

Problem #5 ADL / Functional Status: Current Status: ADL/ IADL: Activities:

LTG Achieve by Dec 31, 2025.

ADL Improvements In: Activities:

- Carrying
 - Dressing
 - Housework- Light
 - Lifting
 - Reaching
 - Sleeping
- 7 - Complete independence
 7 - Complete independence
 7 - Complete independence
 7 - Complete independence
 7 - Complete independence
 7 - Complete independence

Plan**Left Shoulder**

Therapeutic Contents: Therex (97110), Neuroreed (97112), Manual(97140), Therapeutic Act (97530), Self care/Home (97535), Gait (97116), Group (97150), Heat/ice (97010), Estim (97014), US (97035), Dry needling (20560/20561)

Amount, Frequency and Duration: Frequency and Duration: It is recommended that the patient attend rehabilitative therapy for 3 visits a week with an expected duration of 12 weeks. The outlined therapeutic procedures and services in the plan of care will address the problems and goals identified.



Kendell . Baylor, PT, PT(CO Lic: 0019828)
Signed on Sep 29, 2025 22:47:12



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Oct 30, 2025
FSC: HMO/PPO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 17
Cxl/Ns: 6

Re-Evaluation

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

	Previous Sep 29, 2025	Current Oct 30, 2025
• Current	3/10	^a 3/10
• At Worst	4/10	^a 4/10

^a Measured On Sep 02, 2025

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

	Previous Sep 29, 2025	Current Oct 30, 2025
• Date:	09/02/2025	^a 09/02/2025
• Assessment	Initial	^a Initial
• Disability/Symptom Score	77.00	^a 77.00

^a Measured On Sep 02, 2025

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

	Previous Sep 29, 2025	Current Oct 30, 2025
• Carrying	unable	1#

• Dressing	unable to don/doff shirts independently	difficulty full overhead and unable to reach behind his back
• Housework- Light	unable	3 - Moderate assistance
• Lifting	unable	1#
• Personal Hygiene Requirements	unable to clean axillary area	6 - Modified independence
• Reaching	unable	to head
• Sleeping	spasms preventing him from falling asleep	^a spasms preventing him from falling asleep

^a Measured On Sep 02, 2025

Daily Comments: 9/4: was very sore after IE, it's a good sore/ache not painful/sharp in a bad way. Has been compliant with HEP and sling use. Has been trying to touch his stomach which is improving.

9/5: Pt reports the shoulder is still sore but the ice helps

9/8: Pt feeling really stiff today; elbow bends feel really good

9/11: Patient reports that his shoulder is tight.

9/29: Pt reports the shoulder is doing better but the bicep is still the most painful

9/30: Patient states he stopped doing the pullies. Patient reports that he is sore from yesterday.

10/3: Patient reports that Dr. Grey was pleased with everything, that he is stiff, but thats to be expected. Patient reports that his shoulder is feeling good. Patient reports that Dr. Grey wants him out of the sling and we can start pushing it a little bit. He wants him to use the arm but not to pain. We can also start stretching into IR.

10/6: Pt reports high levels of pain and soreness in the shoulder still today following Fridays appointment.

10/7: Patient states that he slept run when sleeping, but after learning how to prop it up its better.

10/9: Patient reports that he got his sleep figured out so now his shoulder is feeling much better. Patient states he did well following Tuesdays appointment.

10/21: The patient reports his shoulder is a little more stiff due to being in the hospital for about a week. He had some sinus issues that caused him to need to be hospitalized.

10/22: Pt reports doing well today and liking the ball on wall exercises added yesterday

10/24: Patient reports that his shoulder is sore, but good.

10/28: The patient reports his shoulder is feeling like it is moving better and he thinks that the ball on the wall exercise is making it easier to turn the steering wheel of his car.

10/30: The patient reports he saw his doctor yesterday and was told he is progressing very well. He states they told him to really focus on ROM and is not allowed to do weights yet.

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

	Previous Sep 29, 2025		Current Oct 30, 2025	
	Left	Right	Left	Right
• Inferior Humeral Glide	Hypomobile/Painful	Normal	^a Hypomobile/Painful	^a Normal
• Posterior Humeral Glide	Hypomobile/Painful	Normal	^a Hypomobile/Painful	^a Normal

^a Measured On Sep 02, 2025

Muscle Testing: Upper Extremity MMT:

	Previous Sep 29, 2025		Current Oct 30, 2025	
	Left	Right	Left	Right
• Shoulder Abduction:(0= NT)	0/5	5/5	^a 0/5	^a 5/5
• Shoulder Flexion:	0/5	5/5	^a 0/5	^a 5/5
• Shoulder External Rotation:	0/5	5/5	^a 0/5	^a 5/5
• Shoulder Internal Rotation:	0/5	5/5	^a 0/5	^a 5/5

^a Measured On Sep 02, 2025

Range of Motion: Shoulder: Pre-Treatment:

	Previous Sep 29, 2025				Current Oct 30, 2025				Change			
	L. Act.	L. Pas.	R. Act.	R. Pas.	L. Act.	L. Pas.	R. Act.	R. Pas.	L. Act.	Pas.	R. Act.	Pas.
● Flexion(0=NT)	0	105	135	150	120	^b 105	^a 135	^a 150	120			
● Abduction	0	110	135	155	108	^b 110	^a 135	^a 155	108			

• External Rotation - 0 deg						65													
• External Rotation - 90 deg	0	32		35	65	a	0	b 32	a	35	a 65								
• Internal Rotation - 90 deg(not measured due to post-op restriction)	0	0		45	45	a	0	b 0	a	45	a 45								

a Measured On Sep 02, 2025 b Measured On Sep 29, 2025

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 10 Minutes, Description: PROM all ways with AAROM with IR for better relaxation, Charge As: Therapeutic Exercise, Performed by: Sara M Lockwood, PT, 10 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Additional Detail: and scar tissue passage, Time Elapsed: 4 Minutes, Tx Depth: Moderate, Technique: Rolling, Description: UT, LS, pec, delt, shoulder girdle/post cuff, Charge As: Manual Therapy Techniques, Performed by: Sara M Lockwood, PT, 4 Minutes.

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)
- T-Bar Flexion(POW #5)
- T-Bar Press Up(POW #5)
- Wall Walk/Finger Ladder(POW #5)
- Rope & Pulley Flexion

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

Did Not Perform: POW #5

Did Not Perform: POW #5

Did Not Perform: POW #5

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Exercise Activities:Isotonics:

- Isotonic Activity 1

Time Elapsed: 4 Minutes, Description: Jobe 1-6 on ball, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 4 Minutes.

Modalities:

- Cryotherapy

Did Not Perform:

Pt./Family Education:

- Patient Education 1
- Patient Education 2

Did Not Perform:

Did Not Perform:

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Time Elapsed: 4 Minutes, Charge As: Manual Therapy Techniques, Performed by: Sara M Lockwood, PT, 4 Minutes.

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball

Additional Detail: at shoulder height and OH positions, Repetitions: 20, Direction: up/down, side/side, circles CW/CCW, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 3 Minutes.

- Closed Kinetic Chain 2

Repetitions: 3 x 30 sec, Description: quadruped with protraction and whole body perturbations, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 3 Minutes.

- Closed Kinetic Chain 3

Repetitions: 10 ea, Description: PNF D1 and D2 with red therabar, Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

- Closed Kinetic Chain 4

Repetitions: 20, Sets: 2, Description: full plank on table with shoulder taps, Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

Therapy Session Time

- Total Therapy Session Time 38 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	8
• Neuromuscular Reeducation	97112	1.00	14
• Reevaluation	97164	1.00	n/a
• Therapeutic Activities	97530	1.00	4
• Therapeutic Procedure	97110	1.00	10

Assessment

Recommendations: Skilled Intervention: Required To: Decrease Pain.

The patient was educated regarding their diagnosis, prognosis, related pathology & plan of care. The patient demonstrates a good understanding of the risks, benefits, precautions/contraindications, & prognosis of their skilled rehabilitation program.

Recommendations: Skilled Intervention:

- **Required To:** Improve Function. Improve Motor Control. Increase Range of Motion. Increase Strength.

Tolerance: The patient continues to present with gradual improvement in his AROM. He is progressing well with closed chain exercises and proprioceptive exercises at this time. PT will continue to focus on proprioception and closed chain activities prior to adding weight and resistance. He will benefit from continued skilled care for full return to prior level of function.

Problems & Goals

Left Shoulder

Problem #1 Chief Complaint: Pain: Severity:

LTG Achieve by Sep 30, 2025. worst

Symptomatic Improvements: Decreasing Pain: to 2/10.

Problem #2 Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

LTG Achieve by Dec 31, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 35.00

STG Achieve by Nov 01, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 50.00

Problem #3 Muscle Testing: Upper Extremity MMT:

LTG Achieve by Dec 31, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

5/5

STG Achieve by Nov 01, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

+3/5

Problem #4 Range of Motion: Shoulder: Pre-Treatment:

LTG Achieve by Dec 31, 2025.

Range of Motion Improvements to: Shoulder:

	Left AROM	Left PROM
• Flexion	145	160
• Abduction	145	160

• External Rotation - 90 deg	85	90
• Internal Rotation - 90 deg	45	50

STG Achieve by Nov 13, 2025. Progress: Good progress.

Range of Motion Improvements to: Shoulder:

	Left AROM	Left PROM
• Flexion	120	145
• Abduction	125	145
• External Rotation - 90 deg	70	90
• Internal Rotation - 90 deg	25	35

Problem #5 ADL / Functional Status: Current Status: ADL/ IADL: Activities:

LTG Achieve by Dec 31, 2025. Progress: Some progress.

ADL Improvements In: Activities:

• Carrying	7 - Complete independence
• Dressing	7 - Complete independence
• Housework- Light	7 - Complete independence
• Lifting	7 - Complete independence
• Reaching	7 - Complete independence
• Sleeping	7 - Complete independence

Plan

Left Shoulder

Therapeutic Contents: Therex (97110), Neuroreed (97112), Manual(97140), Ther Act (97530), Self care/Home (97535), Gait (97116), Dry needling (20560/20561), Heat/ice (97010), Estim (97014), US (97035), Traction (97012)

Amount, Frequency and Duration: Frequency and Duration: It is recommended that the patient attend rehabilitative therapy for 3 visits a week with an expected duration of 8 weeks . The outlined therapeutic procedures and services in the plan of care will address the problems and goals identified.



Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
Signed on Oct 31, 2025 06:46:24



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 04, 2025
FSC: HMO/PPO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 30
Cxl/Ns: 7

Re-Evaluation

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill Sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

	Previous Oct 30, 2025	Current Dec 04, 2025
• Current	3/10	^a 3/10
• At Worst	4/10	^a 4/10

^a Measured On Sep 02, 2025

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

	Previous Oct 30, 2025	Current Dec 04, 2025
• Date:	09/02/2025	^a 09/02/2025
• Assessment	Initial	^a Initial
• Disability/Symptom Score	77.00	^a 77.00

^a Measured On Sep 02, 2025

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

	Previous Oct 30, 2025	Current Dec 04, 2025
• Carrying	1#	3#

• Dressing	difficulty full overhead and unable to reach behind his back	difficulty reaching behind back
• Housework- Light	3 - Moderate assistance	^b 3 - Moderate assistance
• Lifting	1#	3#
• Personal Hygiene Requirements	6 - Modified independence	^b 6 - Modified independence
• Reaching	to head	independent
• Sleeping	spasms preventing him from falling asleep	^a spasms preventing him from falling asleep

a Measured On Sep 02, 2025 b Measured On Oct 30, 2025

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
 11/14: Patient reports that he is sore and the biceps are hurting still.
 11/18: Patient reports that his shoulder feels better and the biceps is not hurting today.
 11/20: The patient reports his shoulder is feeling pretty good and he was not too sore after the last session.
 11/24: Pt reports feeling weird since the pop last session but feels like he gained more mobility with it
 11/25: Patient reports that he is feeling better after last session.
 11/28: Patient reports that his shoulder feels okay.
 12/2: The patient reports his shoulder is feeling less sore and the motion continues to improve.
 12/4: The patient reports his shoulder felt really good after the last session and he was not too sore. It is continuing to feel stronger

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

	Previous Oct 30, 2025		Current Dec 04, 2025	
	Left	Right	Left	Right
• Inferior Humeral Glide	Hypomobile/Painful	Normal	^a Hypomobile/Painful	^a Normal
• Posterior Humeral Glide	Hypomobile/Painful	Normal	^a Hypomobile/Painful	^a Normal

a Measured On Sep 02, 2025

Muscle Testing: Upper Extremity MMT:

	Previous Oct 30, 2025		Current Dec 04, 2025		Change	
	Left	Right	Left	Right	Left	Right
• Shoulder Abduction:(0= NT)	0/5	5/5	-4/5	^a 5/5	Inc'd	
• Shoulder Flexion:	0/5	5/5	4/5	^a 5/5	Inc'd	
• Shoulder External Rotation:	0/5	5/5	4/5	^a 5/5	Inc'd	
• Shoulder Internal Rotation:	0/5	5/5	4/5	^a 5/5	Inc'd	

a Measured On Sep 02, 2025

Range of Motion: Shoulder: Pre-Treatment:

	Previous Oct 30, 2025				Current Dec 04, 2025				Change		
	L. Act.	L. Pas.	R. Act.	R. Pas.	L. Act.	L. Pas.	R. Act.	R. Pas.	L. Act.	Pas.	R. Act.
• Flexion(0=NT)	120	105	135	150	146	^b 105	^a 135	^a 150	26		
• Abduction	108	110	135	155	108	^b 110	^a 135	^a 155			
• External Rotation - 0 deg	65				^c 65						
• External Rotation - 90 deg	0	32	35	65	80	^b 32	^a 35	^a 65	80		
• Internal Rotation - 90 deg(not measured due to post-op restriction)	0	0	45	45	24	^b 0	^a 45	^a 45	24		

a Measured On Sep 02, 2025 b Measured On Sep 29, 2025 c Measured On Oct 30, 2025

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 8 Minutes, Description: PROM all ways, Charge As:
 Therapeutic Exercise, Performed by: Sara M Lockwood, PT, 8 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Did Not Perform:

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)
- Wall Walk/Finger Ladder(with leading in for over pressure stretch)
- Rope & Pulley Flexion
- ROM Activity 1

Time Elapsed: 1 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 1 Minutes.

Did Not Perform: with leading in for over pressure stretch

Time Elapsed: 1 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 1 Minutes.

Technique: Active, Description: sidelying abd, Time Elapsed: 2 Minutes, Charge As: Therapeutic Exercise, Performed by: Sara M Lockwood, PT, 2 Minutes.

Exercise Activities:Isotonics:

Pt./Family Education:

- Patient Education 1
- Patient Education 2

Did Not Perform:

Did Not Perform:

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Technique 2: flex MWM inferior glides, Technique 3: regular inferior glides passive, Time Elapsed: 4 Minutes, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Sara M Lockwood, PT, 4 Minutes.

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball

Time Elapsed: 3 Minutes, Performed: GMB, Additional Detail: at shoulder height in abduction position and OH positions, Repetitions: 30 seconds each way, Direction: up/down, side/side, circles CW/CCW, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 3 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Additional Detail: x10 hold 10 seconds with orange band, Description: quadruped shoulder protraction, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 3 Minutes.

Exercise Activities: Aerobic Conditioning:

- Upper Body Ergometer

Direction: forward & backward, Speed (rpm): self selected, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 4 Minutes.

Exercise Activities: Stabilization Training: General:

- Stabilization Training 1:

Description: BOSU full plank position-side/side, fwd/back, circles CW/CCW, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 4 Minutes.

Did Not Perform:

- Stabilization Training 3:
- Stabilization Training 4:

Description: quadruped with horizontal abd using YTB x 15, Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

Exercise Activities: Tubing/Bands:

Exercise Activities:Machines/Weights:

- Low Rows
Repetitions: 15, Sets: 2, Plates: 3, Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.
- Pull Downs: Sym. Bilateral
Repetitions: 10, Sets: 2, Plates: 4, Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.
- Machines/Free Weights 1
Repetitions: 8, Sets: 2, Description: standing punches with serratus press on cable machine, Plates: 2, Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

Therapy Session Time

- Total Therapy Session Time 38 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	4
• Neuromuscular Reeducation	97112	2.00	19
• Reevaluation	97164	1.00	n/a
• Therapeutic Activities	97530	1.00	5
• Therapeutic Procedure	97110	1.00	10

Assessment

Recommendations: Skilled Intervention: Required To: Decrease Pain.

The patient was educated regarding their diagnosis, prognosis, related pathology & plan of care. The patient demonstrates a good understanding of the risks, benefits, precautions/contraindications, & prognosis of their skilled rehabilitation program.

Recommendations: Skilled Intervention:

- **Required To:** Improve Function. Improve Motor Control. Increase Range of Motion. Increase Strength.

Tolerance: The patient presents with continued improvements in his shoulder strength and AROM however has weakness with abduction. PT will continue to progress per protocol and continue to increase strength as able for full return to prior level of function.

Problems & Goals

Left Shoulder

Problem #1 Chief Complaint: Pain: Severity:

LTG Achieve by Sep 30, 2025. worst

Symptomatic Improvements: Decreasing Pain: to 2/10.

Problem #2 Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

LTG Achieve by Dec 31, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 35.00

STG Achieve by Nov 01, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 50.00

Problem #3 Muscle Testing: Upper Extremity MMT:

LTG Achieve by Dec 31, 2025. Progress: Good progress.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

5/5

STG Goal Achieved Dec 04, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left

- Gross Assessment

+3/5

Problem #4 Range of Motion: Shoulder: Pre-Treatment:

LTG Achieve by Dec 31, 2025. Progress: Good progress.

Range of Motion Improvements to: Shoulder:

- Flexion
- Abduction
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM
145	160
145	160
85	90
45	50

STG Goal Achieved Dec 04, 2025.

Range of Motion Improvements to: Shoulder:

- Flexion
- Abduction
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM
120	145
125	145
70	90
25	35

Problem #5 ADL / Functional Status: Current Status: ADL/ IADL: Activities:

LTG Achieve by Dec 31, 2025. Progress: Good progress.

ADL Improvements In: Activities:

- | | |
|--------------------|---------------------------|
| • Carrying | 7 - Complete independence |
| • Dressing | 7 - Complete independence |
| • Housework- Light | 7 - Complete independence |
| • Lifting | 7 - Complete independence |
| • Reaching | 7 - Complete independence |
| • Sleeping | 7 - Complete independence |

Plan

Left Shoulder

Therapeutic Contents: Therex (97110), Neuroreed (97112), Manual(97140), Ther Act (97530), Self care/Home (97535), Gait (97116), Group (97150), Dry needling (20560/20561), Heat/ice (97010), Estim (97014), US (97035), Traction (97012)

Amount, Frequency and Duration: **Frequency and Duration:** It is recommended that the patient attend rehabilitative therapy for 3 visits a week with an expected duration of 8 weeks. The outlined therapeutic procedures and services in the plan of care will address the problems and goals identified.



Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
 Signed on Dec 04, 2025 17:06:09



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 04, 2025
FSC: HMO/PPO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 30
CxI/Ns: 7

Plan of Care

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

Subjective Examination

Left Shoulder

Chief Complaint: Pain: Severity:

- At Worst 4/10
- Current 3/10

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Assessment Initial
- Date: 09/02/2025
- Disability/Symptom Score 77.00

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Assessment

Recommendations: Skilled Intervention: Required To: Decrease Pain.

The patient was educated regarding their diagnosis, prognosis, related pathology & plan of care. The patient demonstrates a good understanding of the risks, benefits, precautions/contraindications, & prognosis of their skilled rehabilitation program.

Recommendations: Skilled Intervention:

- **Required To:** Improve Function. Improve Motor Control. Increase Range of Motion. Increase Strength.

Tolerance: The patient presents with continued improvements in his shoulder strength and AROM however has weakness with abduction. PT will continue to progress per protocol and continue to increase strength as able for full return to prior level of function.

Problems & Goals

Left Shoulder

Problem #1 Chief Complaint: Pain: Severity:

- At Worst 4/10
- Current 3/10

LTG Achieve by Sep 30, 2025. worst

Symptomatic Improvements: Decreasing Pain: to 2/10.

Problem #2 Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Assessment Initial
- Date: 09/02/2025
- Disability/Symptom Score 77.00

LTG Achieve by Dec 31, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 35.00

STG Achieve by Nov 01, 2025.

Questionnaire Improvements: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Disability/Symptom Score 50.00

Problem #3 Muscle Testing: Upper Extremity MMT:

	Left	Right
• Shoulder Abduction:(0= NT)	-4/5	5/5
• Shoulder External Rotation:	4/5	5/5
• Shoulder Flexion:	4/5	5/5
• Shoulder Internal Rotation:	4/5	5/5

LTG Achieve by Dec 31, 2025. Progress: Good progress.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left
• Gross Assessment
5/5

STG Goal Achieved Dec 04, 2025.

Musculoskeletal Improvements In: Upper Extremity Strength to:

Left
• Gross Assessment
+3/5

Problem #4 Range of Motion: Shoulder: Pre-Treatment:

	Left AROM	Left PROM	Right AROM	Right PROM
• Abduction	108	110	135	155
• External Rotation - 0 deg	65			
• External Rotation - 90 deg	80	32	35	65
• Flexion(0=NT)	146	105	135	150
• Internal Rotation - 90 deg(not measured due to post-op restriction)	24	0	45	45

LTG Achieve by Dec 31, 2025. Progress: Good progress.

Range of Motion Improvements to: Shoulder:

	Left AROM	Left PROM
• Flexion	145	160
• Abduction	145	160
• External Rotation - 90 deg	85	90
• Internal Rotation - 90 deg	45	50

STG Goal Achieved Dec 04, 2025.

Range of Motion Improvements to: Shoulder:

	Left AROM	Left PROM
• Flexion	120	145
• Abduction	125	145
• External Rotation - 90 deg	70	90
• Internal Rotation - 90 deg	25	35

Problem #5 ADL / Functional Status: Current Status: ADL/ IADL: Activities:

• Carrying	3#
• Dressing	difficulty reaching behind back
• Housework- Light	3 - Moderate assistance
• Lifting	3#
• Personal Hygiene Requirements	6 - Modified independence
• Reaching	independent
• Sleeping	spasms preventing him from falling asleep

LTG Achieve by Dec 31, 2025. Progress: Good progress.

ADL Improvements In: Activities:

• Carrying	7 - Complete independence
------------	---------------------------



Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993

Visit Date: Dec 04, 2025

- | | |
|--------------------|---------------------------|
| • Dressing | 7 - Complete independence |
| • Housework- Light | 7 - Complete independence |
| • Lifting | 7 - Complete independence |
| • Reaching | 7 - Complete independence |
| • Sleeping | 7 - Complete independence |

Plan

Left Shoulder

Therapeutic Contents: Therex (97110), Neuroreed (97112), Manual(97140), Ther Act (97530), Self care/Home (97535), Gait (97116), Group (97150), Dry needling (20560/20561), Heat/ice (97010), Estim (97014), US (97035), Traction (97012)

Amount, Frequency and Duration: Frequency and Duration: It is recommended that the patient attend rehabilitative therapy for 3 visits a week with an expected duration of 8 weeks. The outlined therapeutic procedures and services in the plan of care will address the problems and goals identified.

Sara M. Lockwood, PT, PT(CO Lic: PTL0012844)
Signed on Dec 04, 2025 17:06:09

Please Sign and Return

I have reviewed the Plan of Care established for skilled therapy services and certify that the services are required and that they will be provided while the patient is under my care.

Comments/Revisions

Sean Grey

Date

Print Name and Credentials

Patient: Ryan Mack

DOB: Jan 09, 1993

Facility Fax: (970) 561-9091



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
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Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 17, 2025
FSC: HMO/PPD
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 34
Cxl/Ns: 8

Daily Note

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
 11/14: Patient reports that he is sore and the biceps are hurting still.
 11/18: Patient reports that his shoulder feels better and the biceps is not hurting today.
 11/20: The patient reports his shoulder is feeling pretty good and he was not too sore after the last session.
 11/24: Pt reports feeling weird since the pop last session but feels like he gained more mobility with it
 11/25: Patient reports that he is feeling better after last session.
 11/28: Patient reports that his shoulder feels okay.
 12/2: The patient reports his shoulder is feeling less sore and the motion continues to improve.
 12/4: The patient reports his shoulder felt really good after the last session and he was not too sore. It is continuing to feel stronger.
 12/5: Pt reports a good soreness in his L shoulder today after his HEP and last session.
 12/11: The patient reports his shoulder feels a little more stiff in the past day or so.
 12/17: Pt reports no pain in his L shoulder today. Pt states his goal is to improve in the next 2 weeks.

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

- Inferior Humeral Glide
- Posterior Humeral Glide

Left

Hypomobile/Painful
 Hypomobile/Painful

Right

Normal
 Normal

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:(0= NT)
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left

-4/5
 4/5
 4/5
 4/5

Right

5/5
 5/5
 5/5
 5/5

Range of Motion: Shoulder: Pre-Treatment:

- Flexion(0=NT)
- Abduction
- External Rotation - 0 deg
- External Rotation - 90 deg
- Internal Rotation - 90 deg(not measured due to post-op restriction)

Left AROM	Left PROM	Right AROM	Right PROM
146	105	135	150
108	110	135	155
65			
80	32	35	65
24	0	45	45

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 6 Minutes, Description: PROM all ways, Charge As: Therapeutic Exercise, Performed by: Jessica j Uptmore, PTA, 6 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Did Not Perform:

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

- Wall Walk/Finger Ladder(with leading in for over pressure stretch)

Did Not Perform: with leading in for over pressure stretch

- Rope & Pulley Flexion

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

- T-Bar ER 45deg

Repetitions: 5, Hold.: 30 Seconds, Time Elapsed: 3 Minutes, Charge As: Therapeutic Exercise, Performed by: Jessica j Uptmore, PTA, 3 Minutes.

Exercise Activities:Isotonics:

Pt./Family Education:

- Patient Education 1

Did Not Perform:

- Patient Education 2

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball

- Closed Kinetic Chain 1

- Closed Kinetic Chain 2

- Closed Kinetic Chain 3

- Closed Kinetic Chain 4

Exercise Activities: Aerobic Conditioning:

- Upper Body Ergometer

Exercise Activities: Stabilization Training: General:

- Stabilization Training 1:

- Stabilization Training 3:

- Stabilization Training 4:

Exercise Activities: Tubing/Bands:

Exercise Activities: Machines/Weights:

- Low Rows

Exercise Activities: Dynamic Training: TRX:

- TRX Customized Exercise 1

Therapy Session Time

- Therapy Session Start Time
- Therapy Session Stop Time
- Total Therapy Session Time

2025/12/17 15:00:00

12/17/2025 03:30 PM

30 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	6
• Neuromuscular Reeducation	97112	1.00	10
• Therapeutic Activities	97530	1.00	5
• Therapeutic Procedure	97110	1.00	9

Assessment

Pt is improving his shoulder ROM and is still working on his flexion and IR for functional mobility. Pt performed planks on the bosu and prone ball walkouts today to increase his strength and stability. Pt continues to benefit from skilled PT to progress functional ADL's to return to PLOF.

Did Not Perform:

Technique 2: flex MWM inferior glides, Technique 3: regular inferior glides passive, Time Elapsed: 6 Minutes, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Jessica j Uptmore, PTA, 6 Minutes.

Time Elapsed: 1 Minutes, Performed: GMB, Additional Detail: at shoulder height in abduction position and OH positions, Repetitions: 30 seconds each way, Direction: up/down, side/side, circles CW/CCW, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 1 Minutes.

Time Elapsed: 2 Minutes, Repetitions: 10, Description: prone on ball walkouts/inchworms, Charge As: Therapeutic Activities, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Did Not Perform:

Time Elapsed: 2 Minutes, Repetitions: 10 ea, Description: full plank on BOSU (ball side down) with slow mountain climber motion, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Did Not Perform:

Did Not Perform:

Description: BOSU full plank position-side/side, fwd/back, circles CW/CCW, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 3 Minutes.

Did Not Perform:

Did Not Perform:

Time Elapsed: 2 Minutes, Repetitions: 15, Sets: 2, Plates: 4, Charge As: Neuromuscular Reeducation, Performed by: Jessica j Uptmore, PTA, 2 Minutes.

Repetitions: 15, Additional Detail: TRX Row, T, Time Elapsed: 1 Minutes, Charge As: Therapeutic Activities, Performed by: Jessica j Uptmore, PTA, 1 Minutes.

Plan**Left Shoulder****Daily Plan:** Continue w/ Current Rehabilitation Program.

Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
Signed on Dec 18, 2025 09:00:59

Jessica j. Uptmore, PTA, PTA(CO Lic: PTA.0015772)
Signed on Dec 17, 2025 20:52:35



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 18, 2025
FSC: HMO/PTO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 35
Cxl/Ns: 8

Daily Note

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
 11/14: Patient reports that he is sore and the biceps are hurting still.
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 12/5: Pt reports a good soreness in his L shoulder today after his HEP and last session.
 12/11: The patient reports his shoulder feels a little more stiff in the past day or so.
 12/17: Pt reports no pain in his L shoulder today. Pt states his goal is to improve in the next 2 weeks.
 12/18: The patient reports his shoulder continues to do well, however feels stiff from doing more with exercises. It can still feel weak when he is doing prolonged overhead activities.

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

- Inferior Humeral Glide
- Posterior Humeral Glide

Left

Hypomobile/Painful
 Hypomobile/Painful

Right

Normal
 Normal

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:(0= NT)
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left

-4/5
 4/5
 4/5
 4/5

Right

5/5
 5/5
 5/5
 5/5

Range of Motion: Shoulder: Pre-Treatment:

- Flexion(0=NT)
- Abduction
- External Rotation - 0 deg
- External Rotation - 90 deg
- Internal Rotation - 90 deg(not measured due to post-op restriction)

Left AROM	Left PROM	Right AROM	Right PROM
146	105	135	150
108	110	135	155
65			
80	32	35	65
24	0	45	45

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 8 Minutes, Description: PROM all ways, Charge As:
 Therapeutic Exercise, Performed by: Sara M Lockwood, PT, 8 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Did Not Perform:

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by:
 Sara M Lockwood, PT, 2 Minutes.

- Wall Walk/Finger Ladder(with leading in for over pressure stretch)

Did Not Perform: with leading in for over pressure stretch

- Rope & Pulley Flexion

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by:
 Sara M Lockwood, PT, 2 Minutes.

- T-Bar ER 45deg

Did Not Perform:

Exercise Activities:Isotonics:

- Isotonic Activity 2

Time Elapsed: 3 Minutes, Repetitions: 10x, Description: wall clocks with RTB, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 3 Minutes.

Pt./Family Education:

- Patient Education 1
- Patient Education 2

Did Not Perform:

Did Not Perform:

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Technique 2: flex MWM inferior glides, Technique 3: regular inferior glides passive, Time Elapsed: 4 Minutes, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Sara M Lockwood, PT, 4 Minutes.

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball

Time Elapsed: 2 Minutes, Performed: GMB, Additional Detail: at shoulder height in abduction position and OH positions, Repetitions: 30 seconds each way, Direction: up/down, side/side, circles CW/CCW, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Did Not Perform:

Did Not Perform:

- Closed Kinetic Chain 1
- Closed Kinetic Chain 3

Exercise Activities: Aerobic Conditioning:

- Upper Body Ergometer

Direction: forward & backward, Speed (rpm): self selected, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 4 Minutes.

Exercise Activities: Stabilization Training: General:

- Stabilization Training 1:

Description: BOSU full plank position-side/side, fwd/back, circles CW/CCW, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 3 Minutes.

Did Not Perform:

Time Elapsed: 2 Minutes, Description: quadruped in bird dog position with horizontal abd using YTB 2 x 10, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

- Stabilization Training 3:
- Stabilization Training 4:

Exercise Activities: Tubing/Bands:

Exercise Activities: Machines/Weights:

- Low Rows
- Pull Downs: Sym. Bilateral
- Machines/Free Weights 1

Did Not Perform:

Time Elapsed: 1 Minutes, Repetitions: 10, Sets: 2, Plates: 5, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 1 Minutes.

Time Elapsed: 2 Minutes, Repetitions: 10, Sets: 2, Description: standing punches with serratus press on cable machine, Plates: 3, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Exercise Activities: Dynamic Training: TRX:

- TRX Customized Exercise 1

Repetitions: 15, Time Elapsed: 3 Minutes, Additional Detail: TRX Row, T, Y, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 3 Minutes.

Therapy Session Time

- Total Therapy Session Time 36 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	4
• Neuromuscular Reeducation	97112	2.00	17
• Therapeutic Activities	97530	1.00	7
• Therapeutic Procedure	97110	1.00	8

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993

Visit Date Dec 18, 2025

Assessment

Tolerance: The patient presents with continued improvements in his shoudler ROM, particularly into IR. His strength is gradually improving as we are progressing strengthening activities. PT will continue to address his impairments for full return to prior level of function.

Plan

Left Shoulder

Daily Plan: Continue w/ Current Rehabilitation Program.



Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
Signed on Dec 18, 2025 12:04:56



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 19, 2025
FSC: HMO/PTO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 36
Cxl/Ns: 8

Daily Note

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
 11/14: Patient reports that he is sore and the biceps are hurting still.
 11/18: Patient reports that his shoulder feels better and the biceps is not hurting today.
 11/20: The patient reports his shoulder is feeling pretty good and he was not too sore after the last session.
 11/24: Pt reports feeling weird since the pop last session but feels like he gained more mobility with it
 11/25: Patient reports that he is feeling better after last session.
 11/28: Patient reports that his shoulder feels okay.
 12/2: The patient reports his shoulder is feeling less sore and the motion continues to improve.
 12/4: The patient reports his shoulder felt really good after the last session and he was not too sore. It is continuing to feel stronger.
 12/5: Pt reports a good soreness in his L shoulder today after his HEP and last session.
 12/11: The patient reports his shoulder feels a little more stiff in the past day or so.
 12/17: Pt reports no pain in his L shoulder today. Pt states his goal is to improve in the next 2 weeks.
 12/18: The patient reports his shoulder continues to do well, however feels stiff from doing more with exercises. It can still feel weak when he is doing prolonged overhead activities.

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

- Inferior Humeral Glide
- Posterior Humeral Glide

Left

Hypomobile/Painful
 Hypomobile/Painful

Right

Normal
 Normal

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:(0= NT)
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left

-4/5
 4/5
 4/5
 4/5

Right

5/5
 5/5
 5/5
 5/5

Range of Motion: Shoulder: Pre-Treatment:

- Flexion(0=NT)
- Abduction
- External Rotation - 0 deg
- External Rotation - 90 deg
- Internal Rotation - 90 deg(not measured due to post-op restriction)

Left AROM	Left PROM	Right AROM	Right PROM
146	105	135	150
108	110	135	155
65			
80	32	35	65
24	0	45	45

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 8 Minutes, Description: PROM all ways, Charge As: Therapeutic Exercise, Performed by: Miranda E Grey, PTA, 8 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Additional Detail: and scar tissue passage, Time Elapsed: 2 Minutes, Tx Depth: Moderate, Technique: Rolling, Description: UT, LS, pec, delt, shoulder girdle/post cuff, Charge As: Manual Therapy Techniques, Performed by: Miranda E Grey, PTA, 2 Minutes.

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)
- Wall Walk/Finger Ladder(with leading in for over pressure stretch)
- Rope & Pulley Flexion

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Miranda E Grey, PTA, 2 Minutes.

Did Not Perform: with leading in for over pressure stretch

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Miranda E Grey, PTA, 2 Minutes.

- T-Bar ER 45deg

Exercise Activities: Isotonics:

- Isotonic Activity 2

Pt./Family Education:

- Patient Education 1
- Patient Education 2

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Technique 2: flex MWM inferior glides, Technique 3: regular inferior glides passive, Time Elapsed: 4 Minutes, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Miranda E Grey, PTA, 4 Minutes.

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball
- Closed Kinetic Chain 1
- Closed Kinetic Chain 3

Did Not Perform:

Did Not Perform:

Did Not Perform:

Exercise Activities: Aerobic Conditioning:

- Upper Body Ergometer

Direction: forward & backward, Speed (rpm): self selected, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Miranda E Grey, PTA, 4 Minutes.

Exercise Activities: Stabilization Training: General:

- Stabilization Training 1:

Description: BOSU full plank position-side/side, fwd/back, circles CW/CCW, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Miranda E Grey, PTA, 3 Minutes.

Did Not Perform:

Did Not Perform:

- Stabilization Training 3:

- Stabilization Training 4:

Exercise Activities: Tubing/Bands:

Exercise Activities: Machines/Weights:

- Low Rows

Additional Detail: single and double, Time Elapsed: 6 Minutes, Repetitions: 15, Sets: 2, Plates: 4/3, Charge As: Neuromuscular Reeducation, Performed by: Miranda E Grey, PTA, 6 Minutes.

Time Elapsed: 1 Minutes, Repetitions: 10, Sets: 2, Plates: 5, Charge As: Neuromuscular Reeducation, Performed by: Miranda E Grey, PTA, 1 Minutes.

Time Elapsed: 2 Minutes, Repetitions: 10, Sets: 2, Description: standing punches with serratus press on cable machine, Plates: 3, Charge As: Neuromuscular Reeducation, Performed by: Miranda E Grey, PTA, 2 Minutes.

- Pull Downs: Sym. Bilateral

- Machines/Free Weights 1

Exercise Activities: Dynamic Training: TRX:

- TRX Customized Exercise 1

Did Not Perform:

Therapy Session Time

- Total Therapy Session Time 34 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	6
• Neuromuscular Reeducation	97112	2.00	18
• Therapeutic Activities	97530	1.00	2
• Therapeutic Procedure	97110	1.00	8

Assessment

Patient shows gains in shoulder stability and strength comparable to past sessions. Patient continues to show decrease limitation with shoulder IR/ER. Patient will continue to benefit from skilled PT and progress per protocol.

Plan**Left Shoulder****Daily Plan:** Continue w/ Current Rehabilitation Program.

Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
Signed on Dec 19, 2025 12:28:31
Signed in the absence of the primary therapist.



Miranda E. Grey, PTA, PTA(CO Lic: PTA.0014877)
Signed on Dec 19, 2025 12:27:59
Under the general supervision of the primary therapist.



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Kendell Baylor, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 22, 2025
FSC: HMO/PPG
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 37
Cxl/Ns: 8

Daily Note

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
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12/18: The patient reports his shoulder continues to do well, however feels stiff from doing more with exercises. It can still feel weak when he is doing prolonged overhead activities.
12/22: Pt reports shoulder still feeling stiff, sees Dr Grey tomorrow maybe for his last visit

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

- Inferior Humeral Glide
- Posterior Humeral Glide

Left

Hypomobile/Painful
Hypomobile/Painful

Right

Normal
Normal

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left

-4/5
4/5
4/5
4/5

Right

5/5
5/5
5/5
5/5

Range of Motion: Shoulder: Pre-Treatment:

- Flexion
- Abduction
- External Rotation - 0 deg
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM	Right AROM	Right PROM
125	175	135	150
103	165	135	155
65			
75	95	35	65
30	60	45	45

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 7 Minutes, Description: PROM all ways, Charge As: Therapeutic Exercise, Performed by: Kendell Baylor, PT, 7 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Additional Detail: and scar tissue passage, Time Elapsed: 3 Minutes, Tx Depth: Moderate, Technique: Rolling, Description: UT, LS, pec, delt, shoulder girdle/post cuff, Charge As: Manual Therapy Techniques, Performed by: Kendell Baylor, PT, 3 Minutes.

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)
- Wall Walk/Finger Ladder(with leading in for over pressure stretch)

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Kendell Baylor, PT, 2 Minutes.

Did Not Perform: with leading in for over pressure stretch

- Rope & Pulley Flexion

- T-Bar ER 45deg
- ROM Activity 2

Exercise Activities: Isotonics:

- Isotonic Activity 2

Pt./Family Education:

- Patient Education 1

- Written Home Exercise Program

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball
- Closed Kinetic Chain 1
- Closed Kinetic Chain 3

Exercise Activities: Aerobic Conditioning:

Exercise Activities: Stabilization Training: General:

- Stabilization Training 2: (Pt active assisted help with Y's during concentric phase)
- Stabilization Training 3:
- Stabilization Training 4:

Exercise Activities: Tubing/Bands:

Exercise Activities: Machines/Weights:

- Low Rows

- Machines/Free Weights 1

- Push Downs: Unilateral

- Machines/Free Weights 2

- Machines/Free Weights 3

Exercise Activities: Dynamic Training: TRX:

- TRX Customized Exercise 1

Therapy Session Time

- Therapy Session Start Time
- Therapy Session Stop Time
- Total Therapy Session Time

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 2 Minutes.

Did Not Perform:

Side: Bilateral, Technique: Active Assistive, Description: TRX flex and abd walkout stretching, Time Elapsed: 4 Minutes, Charge As: Therapeutic Exercise, Performed by: Kendell Baylor, PT, 4 Minutes.

Did Not Perform:

Time Elapsed: 2 Minutes, Description: ROM, joint versus mm restriction, goals, POC, Charge As: Therapeutic Activities, Performed by: Kendell Baylor, PT, 2 Minutes.

Activity: Provided & Reviewed, Description: strength and education on adding weight to some interventions, Time Elapsed: 2 Minutes, Charge As: Therapeutic Exercise, Performed by: Kendell Baylor, PT, 2 Minutes.

Time Elapsed: 5 Minutes, Technique 2: flex mobs towards end range, Technique 3: regular inferior glides passive, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Kendell Baylor, PT, 5 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Exercise 1: prone swiss ball I,T,Y (#1lbs for I's and T's), Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 2 Minutes.

Did Not Perform:

Did Not Perform:

Additional Detail: single and double, Time Elapsed: 4 Minutes, Repetitions: 15, Sets: 2, Plates: 4/3, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 4 Minutes.

Time Elapsed: 3 Minutes, Repetitions: 10, Sets: 2, Description: standing punches with serratus press on cable machine, Plates: 3, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 3 Minutes.

Weight - Pounds: #12.5 Pounds, Repetitions: 10, Sets: 2, Additional Detail: seated single arm lat pulldowns with passive flex stretch at top, Time Elapsed: 3 Minutes, Charge As: Therapeutic Activities, Performed by: Kendell Baylor, PT, 3 Minutes.

Weight - Pounds: #5 Pounds, Sets: 2 laps, Description: 90 flex carries, Time Elapsed: 3 Minutes, Charge As: Therapeutic Activities, Performed by: Kendell Baylor, PT, 3 Minutes.

Weight - Pounds: #5 Pounds, Sets: 2 laps, Description: 90 flex carries with 90/90 movement walking, Time Elapsed: 3 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Kendell Baylor, PT, 3 Minutes.

Did Not Perform:

2025/12/22 16:30:00

12/22/2025 05:15 PM

45 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	8
• Neuromuscular Reeducation	97112	1.00	14
• Therapeutic Activities	97530	1.00	10
• Therapeutic Procedure	97110	1.00	13

Assessment

Focused session on assessment of strength and ROM and pushing the AROM and PROM. Educated on strength at home and HEP. Pt tolerated session well with high fatigue during weighted interventions and some intense stretching with joint mobs today. Reported good understanding of education.

Plan***Left Shoulder***

Daily Plan: Continue w/ Current Rehabilitation Program. Advance as tolerated. Progression Under Current Plan.



Kendell . Baylor, PT, PT(CO Lic: 0019828)
Signed on Dec 23, 2025 06:01:54



Select Physical Therapy
1230 West Ash Street
Suite A
Windsor, CO, USA, 80550
Phone: (970) 561-9090
Fax: (970) 561-9091

Patient: Ryan Mack
Acct #: 035R241605911
DOB: Jan 09, 1993
Clinician: Sara M. Lockwood, PT
Prim Phy: Sean Grey
Phy Phone: (970) 493-0112
Phy Fax: (970) 419-7103
Sec Phy: Not Specified
Inj. Date: 8/20/2025
Surg. Date: 8/20/2025

Visit Date: Dec 23, 2025
FSC: HMO/PPO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 38
Cxl/Ns: 8

Daily Note

Diagnosis

Left M25.512 Pain in left shoulder
Shoulder

General Information

History of Injury: Pt is a 32 yr old with c/o of left shoulder pain. Pt reports that their shoulder left dislocated during his sleep in late June. Went to urgent care and he was fine even though he couldn't move his arm at all because it was posterior located. He went home and then they called when they saw the x-ray and said to go to the ER and at the ER he was sedated and they made multiple attempted to relocate. At that point he was put in an immobilizer and told wait to 6 week and then he went to OCR and saw Dr. Grey who told him he needed immediate surgery. He just moved to Windsor because he can't work due to his current injury. Prior to surgery they were not using their arm or moving their arm at all. At their post op appointment they were told they would get back to PLOF in about 6 months. He was given the the PROM Exercise and told to avoid active abduction.

Review of Systems: PMH: upper GI issues; history of R shoulder instability; contraindications to therapy: e-stim due to recent seizure. He has been having myoclonic jerks every evening when trying falling asleep.

Occupation: Pt currently is unable to work but he needs to return to lifting and moving and throwing heavy loads including trees and debris.

Reason for Referral: Referred by Dr. Grey for posterior shoulder instability with large reverse Hill sachs lesion of left shoulder. s/p open modified McLaughlin procedure on 8/20/25,

The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Treatment Guidelines:

- Evaluate and Treat
- Rehabilitative Program

Additional Detail: 2-3x/wk, Duration: 12

Additional Detail: Gentle PROM only; limit aggressive passive IR; ER ok

Patient has been identified as a fall risk at the time of this evaluation therefore requiring general supervision in clinic.

Subjective Examination

Left Shoulder

Chief Complaint: Currently, pt reports moderate pain at most times. Usually a dull ache. He reports notable difficulty with ADLs, pain management and is compliant with sling use and precautions.

Chief Complaint: Pain: Severity:

- Current 3/10
- At Worst 4/10

Mechanism of Injury: Primary Episode: spontaneous seizure during sleep

Questionnaires: C-D: Disabilities of Arm, Shoulder, Hand (DASH/QDASH): QuickDASH Scoring:

- Date: 09/02/2025
- Assessment Initial
- Disability/Symptom Score 77.00

Rehabilitation Expectations/Goals: Eliminate: pt would like to return to climbing trees, snowboarding, skate boarding.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Carrying 3#
- Dressing difficulty reaching behind back
- Housework- Light 3 - Moderate assistance
- Lifting 3#
- Personal Hygiene Requirements 6 - Modified independence
- Reaching independent
- Sleeping spasms preventing him from falling asleep

Daily Comments: 11/13: The patient reports his shoulder is pretty tired from the exercises last session.
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12/22: Pt reports shoulder still feeling stiff, sees Dr Grey tomorrow maybe for his last visit

Objective Examination

Left Shoulder

Joint Integrity/Mobility: Shoulder:

- Inferior Humeral Glide
- Posterior Humeral Glide

Left

Hypomobile/Painful
Hypomobile/Painful

Right

Normal
Normal

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left

-4/5
4/5
4/5
4/5

Right

5/5
5/5
5/5
5/5

Range of Motion: Shoulder: Pre-Treatment:

- Flexion
- Abduction
- External Rotation - 0 deg
- External Rotation - 90 deg
- Internal Rotation - 90 deg

Left AROM	Left PROM	Right AROM	Right PROM
125	175	135	150
103	165	135	155
65			
75	95	35	65
30	60	45	45

Treatments

Left Shoulder

Manual Interventions: Range of Motion: Shoulder:

- Manual ROM 1

Time Elapsed: 6 Minutes, Description: PROM all ways, Charge As: Therapeutic Exercise, Performed by: Sara M Lockwood, PT, 6 Minutes.

Manual Interventions: Soft Tissue:

- Soft Tissue Mobilization 1

Additional Detail: and scar tissue passage, Time Elapsed: 3 Minutes, Tx Depth: Moderate, Technique: Rolling, Description: UT, LS, pec, delt, shoulder girdle/post cuff, Charge As: Manual Therapy Techniques, Performed by: Sara M Lockwood, PT, 3 Minutes.

Exercise Activities: Range of Motion:

- Rope & Pulley Abduction(POW #4)
- Wall Walk/Finger Ladder(with leading in for over pressure stretch)

Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

Did Not Perform: with leading in for over pressure stretch

- Rope & Pulley Flexion

- T-Bar ER 45deg
- ROM Activity 2

Exercise Activities: Isotonics:

- Isotonic Activity 2

Pt./Family Education:

- Patient Education 1
- Written Home Exercise Program

Manual Interventions: Joint Segments: Shoulder:

- Glenohumeral Joint

Exercise Activities: Dynamic Training: C-K-C Activity:

- Wall Ball

- Closed Kinetic Chain 1
- Closed Kinetic Chain 3

Exercise Activities: Aerobic Conditioning:

- Upper Body Ergometer

Exercise Activities: Stabilization Training: General:

- Stabilization Training 2: (Pt active assisted help with Y's during concentric phase)
- Stabilization Training 3:
- Stabilization Training 4:

Exercise Activities: Tubing/Bands:

Exercise Activities: Machines/Weights:

- Low Rows

- Pull Downs: Sym. Bilateral

- Machines/Free Weights 1

- Push Downs: Unilateral
- Machines/Free Weights 2

- Machines/Free Weights 3

Exercise Activities: Dynamic Training: TRX:

- TRX Customized Exercise 1

Time Elapsed: 2 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Time Elapsed: 5 Minutes, Technique 2: flex mobs towards end range, Technique 3: regular inferior glides passive, Grade: II/III, Pressure: Oscillatory, Technique 1: PAs, Charge As: Manual Therapy Techniques, Performed by: Sara M Lockwood, PT, 5 Minutes.

Time Elapsed: 2 Minutes, Performed: GMB, Additional Detail: at shoulder height in abduction position and OH positions, Repetitions: 30 seconds each way, Direction: up/down, side/side, circles CW/CCW, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Did Not Perform:

Did Not Perform:

Direction: forward & backward, Speed (rpm): self selected, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 4 Minutes.

Did Not Perform: Pt active assisted help with Y's during concentric phase

Did Not Perform:

Did Not Perform:

Additional Detail: single and double, Time Elapsed: 4 Minutes, Repetitions: 15, Sets: 2, Plates: 4/3, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 4 Minutes.

Time Elapsed: 2 Minutes, Repetitions: 10, Sets: 2, Plates: 5, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Time Elapsed: 2 Minutes, Repetitions: 10, Sets: 2, Description: standing punches with serratus press on cable machine, Plates: 3, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 2 Minutes.

Did Not Perform:

Weight - Pounds: #5 Pounds, Sets: 2 laps, Description: 90 flex carries, Time Elapsed: 1 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 1 Minutes.

Weight - Pounds: #5 Pounds, Sets: 2 laps, Description: 90 flex carries with 90/90 movement walking, Time Elapsed: 1 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Sara M Lockwood, PT, 1 Minutes.

Repetitions: 15, Additional Detail: TRX Row, T, Y, Time Elapsed: 2 Minutes, Charge As: Therapeutic Activities, Performed by: Sara M Lockwood, PT, 2 Minutes.

Therapy Session Time

- Total Therapy Session Time 36 Minutes

Documented Procedural Code Summary:

Description	Code	Units	Minutes
• Manual Therapy Techniques	97140	1.00	8
• Neuromuscular Reeducation	97112	2.00	17
• Therapeutic Activities	97530	1.00	5
• Therapeutic Procedure	97110	1.00	6

Assessment

Tolerance: The patient presents with improvements in his overall strength with progression in physical therapy. He exhibits continued improvements in his shoulder ROM with PROM. PT will continue to have increased focus on strengthening for participation in his daily activities.

Plan

Left Shoulder

Daily Plan: Continue w/ Current Rehabilitation Program.



Sara M. Lockwood, PT, PT(CO Lic: PTL.0012844)
Signed on Dec 23, 2025 16:50:24